

FIJI NATIONAL JOINT REPLACEMENT REGISTRY

Hip, Knee & Revision Joint Replacement

Annual Report 2025

Procedures recorded 2016–2025. Revision outcomes to 31/12/2025.

FNJRR — monitoring joint replacement activity, implant use and outcomes in Fiji.

Generated 03/11/2026. Contains aggregate, de-identified statistics only.

Synthetic demonstration data — not real patients or surgeons

Summary 2025

In 2025, the registry recorded 213 joint replacement procedures: 121 hip and 92 knee procedures. This is 5 more than in 2024 (208). 208 were primary procedures and 5 were revisions. Since the registry began in 2016, 1,473 procedures have been recorded.

PROCEDURES
213
208 in 2024

HIP
121
116 in 2024

KNEE
92
92 in 2024

REVISIONS
5
10 in 2024

HOSPITALS
5
contributing in 2025

Table 1 Procedures in 2025 by type

Procedure	2024	2025	Change	All years
Primary total hip replacement	70	69	-1	471
Primary hip hemiarthroplasty	42	47	+5	277
Primary total knee replacement	76	83	+7	591
Primary partial knee replacement	10	9	-1	53
Revision hip replacement	4	5	+1	40
Revision knee replacement	6	0	-6	41
Total	**208**	**213**	**+5**	**1,473**

Key findings

Primary total hip replacement. There were 69 primary total hip replacements in 2025, and 471 in total since 2016. Patients had a mean age of 64.8 years and 42.0% were women. The most common diagnosis was osteoarthritis (59.4%). At 5 years, 4.8% of these joints had been revised. The most common reason for revision was dislocation / instability.

Primary hip hemiarthroplasty. There were 47 hip hemiarthroplastys in 2025, and 277 in total since 2016. Patients had a mean age of 78.0 years and 66.0% were women. The most common diagnosis was fractured neck of femur (100.0%). At 5 years, 6.8% of these joints had been revised. The most common reason for revision was periprosthetic fracture.

Primary total knee replacement. There were 83 primary total knee replacements in 2025, and 591 in total since 2016. Patients had a mean age of 66.4 years and 69.9% were women. The most common diagnosis

was osteoarthritis (86.7%). At 5 years, 4.4% of these joints had been revised. The most common reason for revision was loosening.

Primary partial knee replacement. There were 9 primary partial knee replacements in 2025, and 53 in total since 2016. Patients had a mean age of 62.0 years and 0.0% were women. The most common diagnosis was osteoarthritis (100.0%). At 5 years, 4.3% of these joints had been revised. The most common reason for revision was loosening.

Revisions. There were 5 revision procedures in 2025 (5 hip and 0 knee). The most common reason for revision was periprosthetic fracture (80.0%). 100.0% of revisions were linked to a primary procedure in the registry.

1 Registry activity

Figure 1 Hip and knee procedures by year

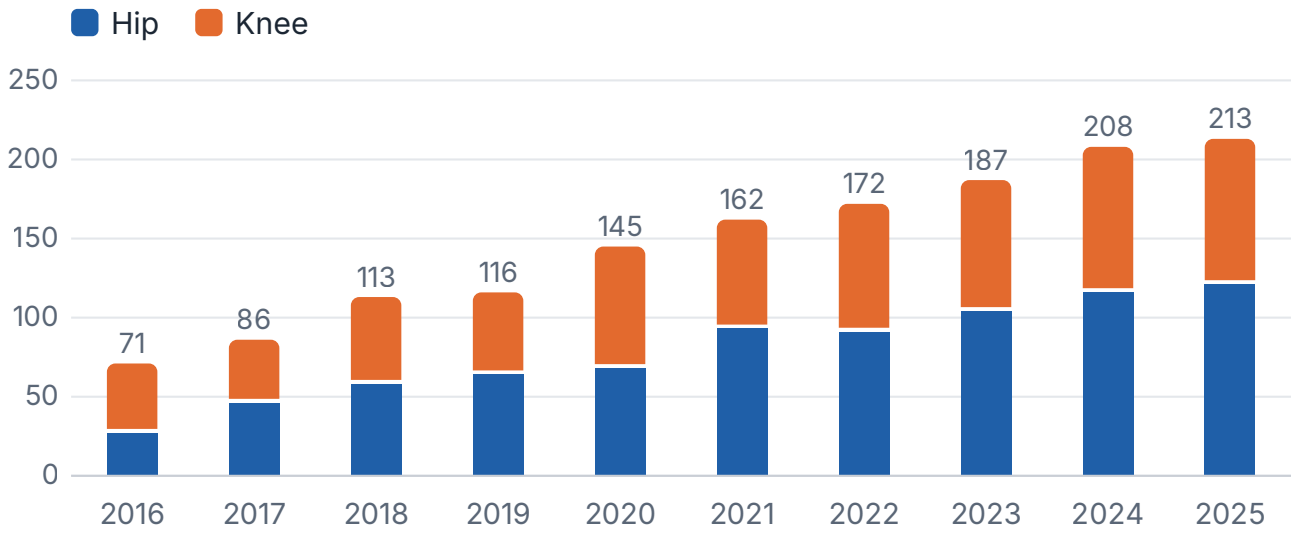


Table 2 Number of procedures by type and year

Procedure	2016	2017	2018	2019	2020	2021	2022	2023	2024	2025	Total
Total hip	13	28	35	37	31	62	58	68	70	69	471
Hip hemiarthroplasty	9	18	22	25	32	24	26	32	42	47	277
Total knee	40	34	43	50	64	59	70	72	76	83	591
Partial knee	3	2	7	1	8	3	5	5	10	9	53
Revision hip	5	0	1	2	5	7	7	4	4	5	40
Revision knee	1	4	5	1	5	7	6	6	6	0	41
Total	**71**	**86**	**113**	**116**	**145**	**162**	**172**	**187**	**208**	**213**	**1,473**

Figure 2 Procedures by hospital, 2025

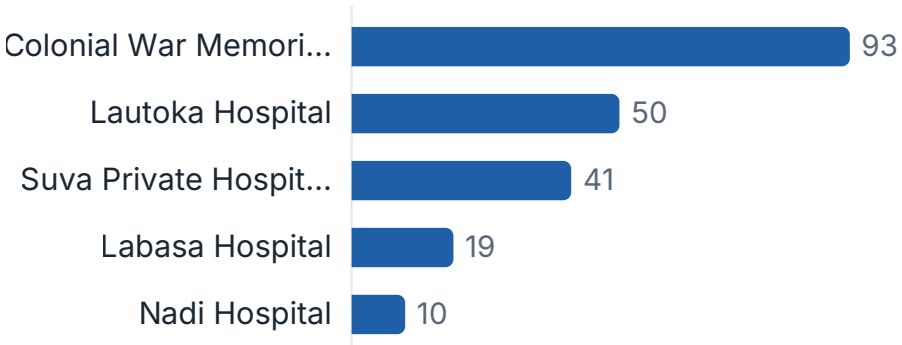


Table 3 Procedures by hospital

Hospital	2024	2025	Change	% of 2025
Colonial War Memorial Hospital	93	93	+0	43.7
Lautoka Hospital	51	50	-1	23.5
Suva Private Hospital	35	41	+6	19.2
Labasa Hospital	14	19	+5	8.9
Nadi Hospital	15	10	-5	4.7

2 Primary total hip replacement

There were 69 primary total hip replacements in 2025, and 471 in total since 2016. Patients had a mean age of 64.8 years and 42.0% were women. The most common diagnosis was osteoarthritis (59.4%). At 5 years, 4.8% of these joints had been revised. The most common reason for revision was dislocation / instability.

Patients in 2025

Table 4 Age and sex, 2025

Age group	Female	Male	Total
<55	9	12	21
55–64	5	7	12
65–74	6	9	15
75–84	6	9	15
≥85	3	3	6
Total	**29**	**40**	**69**

Mean age 64.8 years; median 66.

Table 5 Primary diagnosis

Diagnosis	2025	%	All years	%
Osteoarthritis	41	59.4	290	61.6
Fractured neck of femur	10	14.5	57	12.1
Osteonecrosis (AVN)	7	10.1	51	10.8
Rheumatoid / inflammatory arthritis	5	7.2	30	6.4
Developmental dysplasia	1	1.4	24	5.1
Post-traumatic arthritis	5	7.2	19	4.0

Table 6 Fixation

Fixation	2025	%	All years	%
Cementless	30	43.5	191	40.6
Hybrid (cemented femoral / stem)	19	27.5	177	37.6
Cemented	20	29.0	103	21.9

Table 7 Most used prostheses

Prosthesis	2025	All years
Exeter V40 / Trident	11	107
Exeter V40 / Contemporary	20	103
Corail / Pinnacle	15	87
Accolade II / Trident	8	60
CPT / Continuum	6	50
Polarstem / R3	7	44
Exeter V40 / Latitud	2	20

Revision outcomes (cumulative percent revision)

Figure 3 Cumulative percent revision of primary total hip replacement

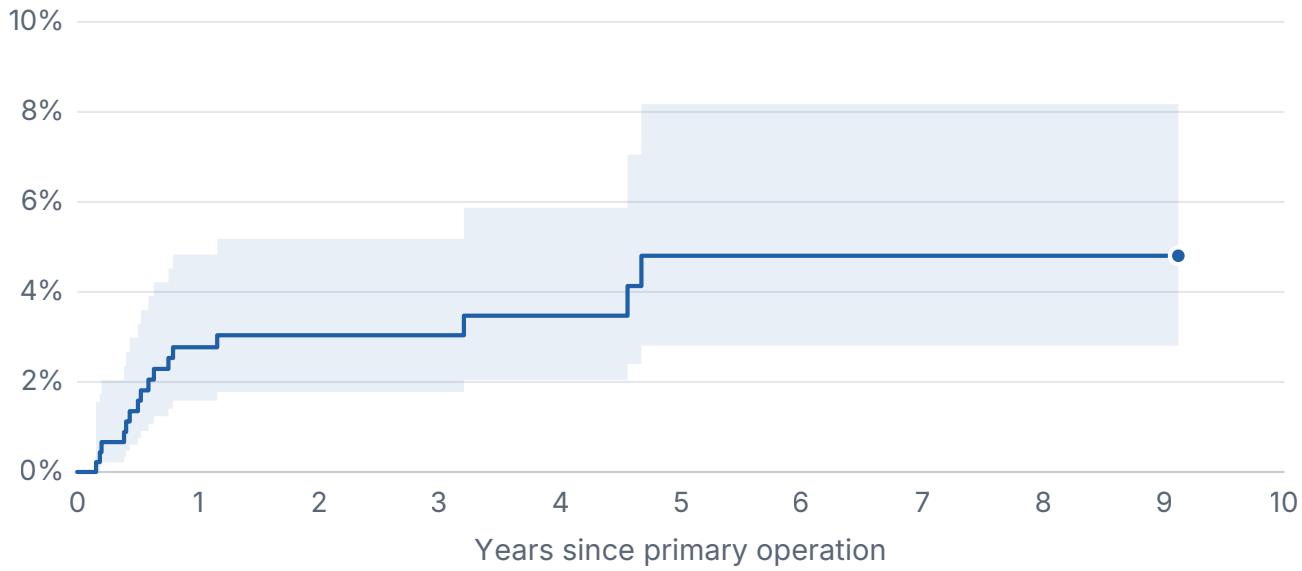


Table 8 Cumulative percent revision of primary total hip replacement (95% CI)

Group	N	N revised	1 yr	3 yrs	5 yrs	10 yrs
All	471	16	2.8 (1.6, 4.8)	3.0 (1.8, 5.2)	4.8 (2.8, 8.2)	—
By fixation						
Cementless	191	10	3.5 (1.6, 7.5)	4.1 (2.0, 8.5)	8.7 (4.4, 16.8)	—
Hybrid (cemented femoral / stem)	177	5	3.0 (1.3, 7.1)	3.0 (1.3, 7.1)	3.0 (1.3, 7.1)	—
Cemented	103	1	1.0 (0.1, 6.8)	1.0 (0.1, 6.8)	1.0 (0.1, 6.8)	—
By age at operation						
<65	265	12	4.0 (2.2, 7.3)	4.0 (2.2, 7.3)	5.6 (3.1, 10.0)	—
65–74	83	3	2.7 (0.7, 10.3)	2.7 (0.7, 10.3)	—	—
≥75	123	1	0.0 (0.0, 0.0)	1.1 (0.2, 7.5)	1.1 (0.2, 7.5)	—

"—" = not enough patients followed up to that time (fewer than 10 at risk, or fewer than 10 procedures in the group).

Figure 4 **Cumulative percent revision of primary total hip replacement by fixation**

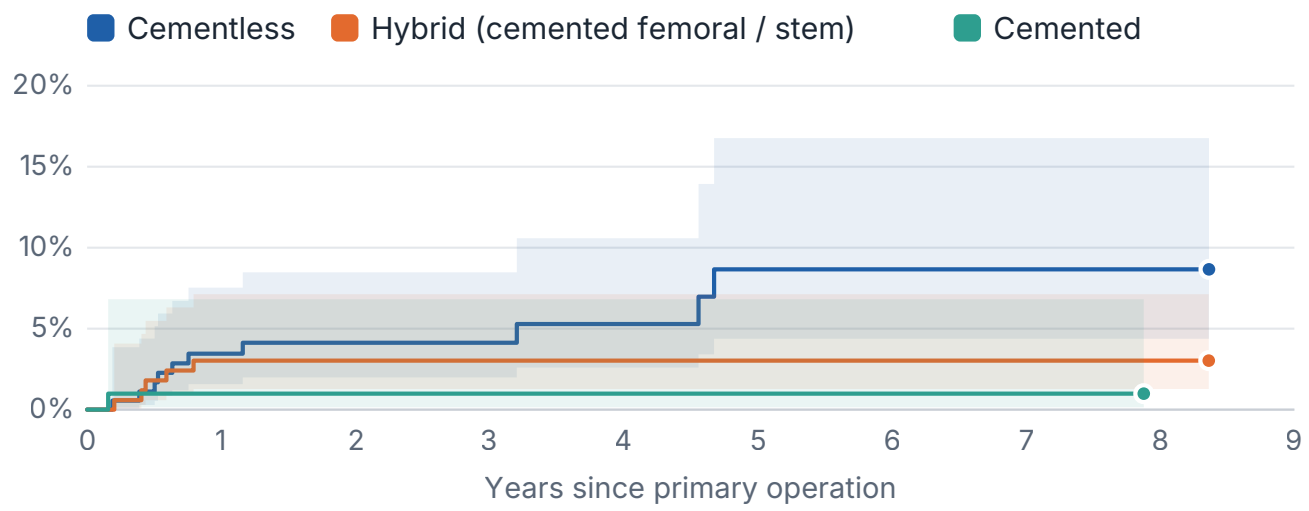


Figure 5 **Cumulative percent revision of primary total hip replacement by age at operation**

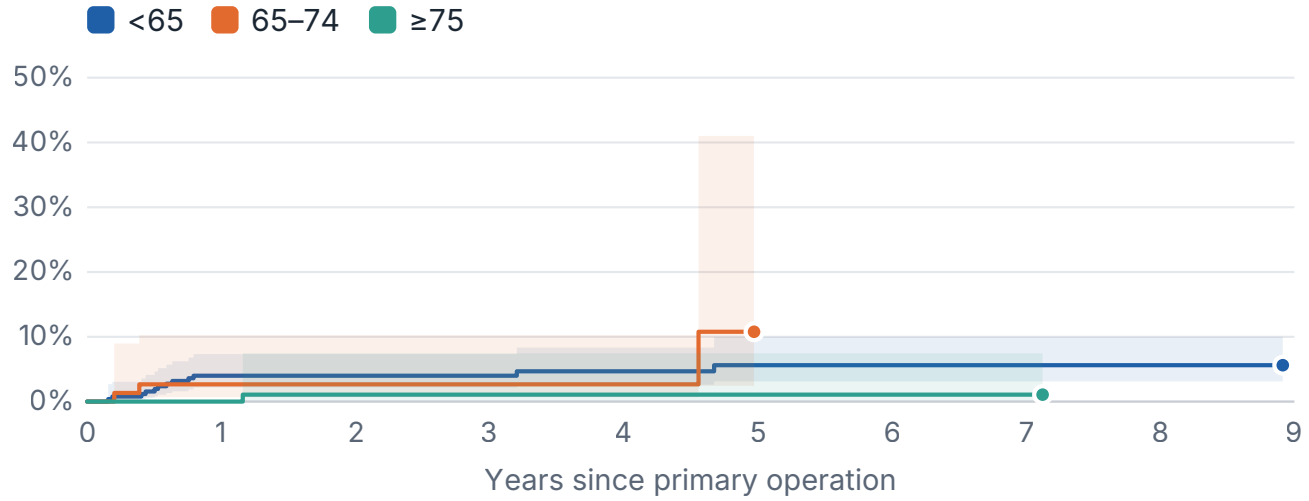


Table 9 **Reasons for revision**

Reason	N	%
Dislocation / instability	5	31.2
Periprosthetic fracture	4	25.0
Loosening	3	18.8
Infection	3	18.8
Other	1	6.2
Total	**16**	**100.0**

Table 10 **Type of revision**

Type	N	%
Major total	6	37.5
Minor	5	31.2
Major partial	5	31.2

3 Primary hip hemiarthroplasty

There were 47 hip hemiarthroplastys in 2025, and 277 in total since 2016. Patients had a mean age of 78.0 years and 66.0% were women. The most common diagnosis was fractured neck of femur (100.0%). At 5 years, 6.8% of these joints had been revised. The most common reason for revision was periprosthetic fracture.

Patients in 2025

Table 11 Age and sex, 2025

Age group	Female	Male	Total
<55	0	0	0
55–64	0	1	1
65–74	14	7	21
75–84	10	3	13
≥85	7	5	12
Total	**31**	**16**	**47**

Mean age 78.0 years; median 77.

Table 12 Primary diagnosis

Diagnosis	2025	%	All years	%
Fractured neck of femur	47	100.0	277	100.0

Table 13 Fixation

Fixation	2025	%	All years	%
Cemented	32	68.1	174	62.8
Cementless	15	31.9	103	37.2

Table 14 Most used prostheses

Prosthesis	2025	All years
Exeter V40 / UHR Bipolar	16	94
Exeter V40 / Exeter Unipolar	16	80
Austin Moore	7	64
Corail / Self-centering bipolar	8	39

Revision outcomes (cumulative percent revision)

Figure 6 Cumulative percent revision of hip hemiarthroplasty

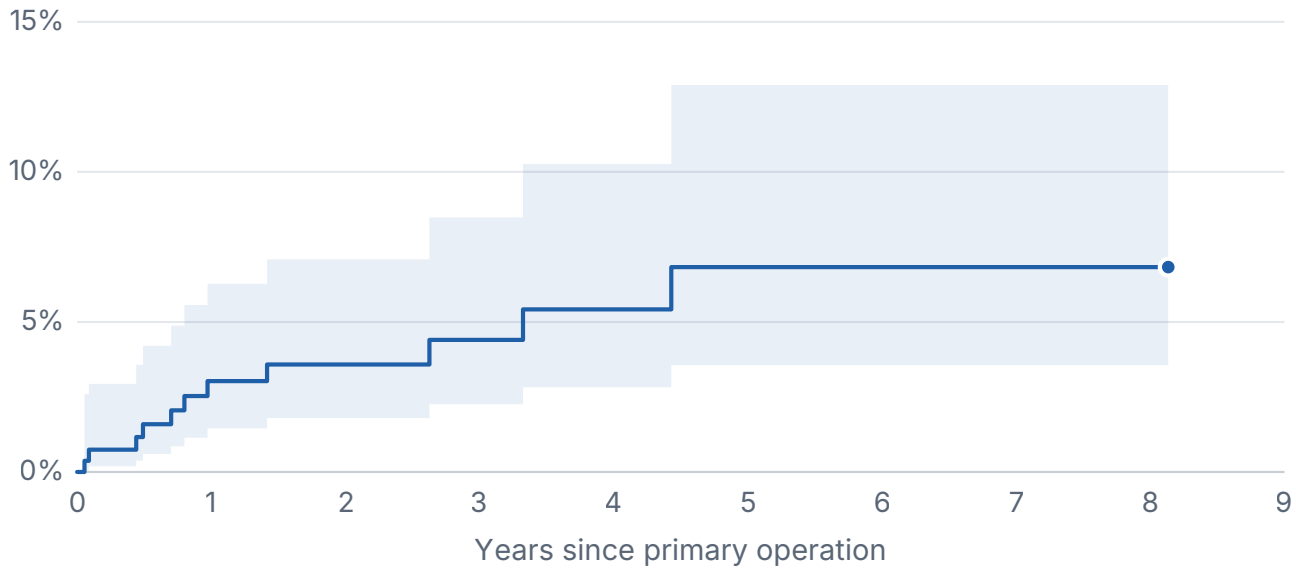


Table 15 Cumulative percent revision of hip hemiarthroplasty (95% CI)

Group	N	N revised	1 yr	3 yrs	5 yrs	10 yrs
All	277	11	3.0 (1.4, 6.3)	4.4 (2.3, 8.5)	6.8 (3.6, 12.9)	—
By fixation						
Cemented	174	8	3.6 (1.5, 8.4)	4.5 (2.0, 9.7)	8.0 (3.8, 16.7)	—
Cementless	103	3	2.2 (0.6, 8.6)	4.5 (1.4, 14.5)	4.5 (1.4, 14.5)	—
By age at operation						
<65	20	0	0.0 (0.0, 0.0)	—	—	—
65–74	87	2	1.4 (0.2, 9.7)	4.4 (1.0, 17.9)	4.4 (1.0, 17.9)	—
≥75	170	9	4.2 (1.9, 9.3)	5.1 (2.5, 10.6)	8.9 (4.3, 17.8)	—

"—" = not enough patients followed up to that time (fewer than 10 at risk, or fewer than 10 procedures in the group).

Figure 7 **Cumulative percent revision of hip hemiarthroplasty by fixation**

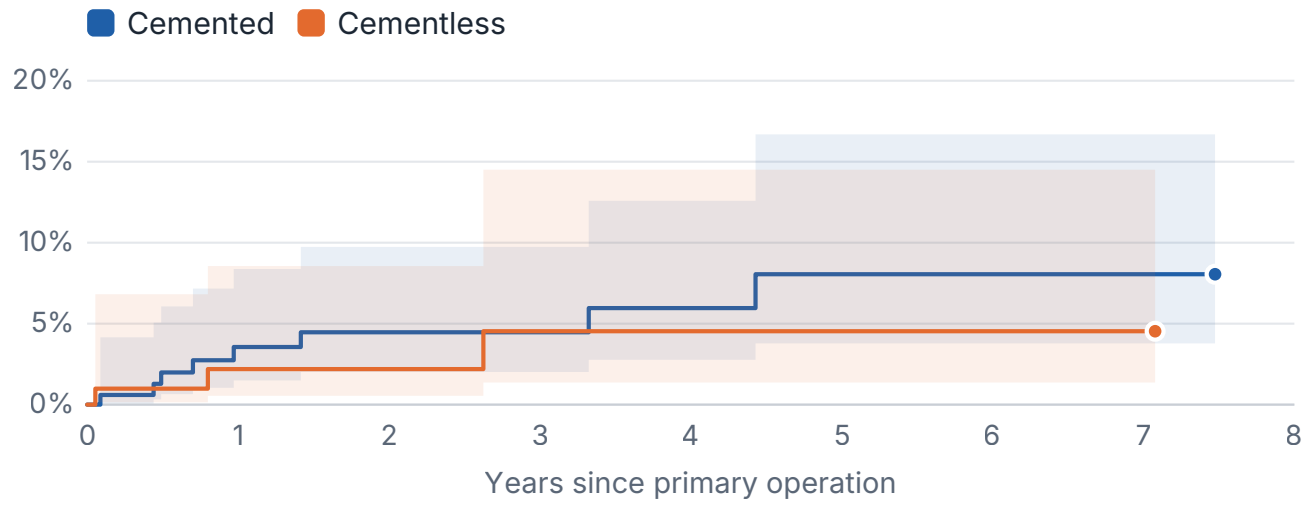


Figure 8 **Cumulative percent revision of hip hemiarthroplasty by age at operation**

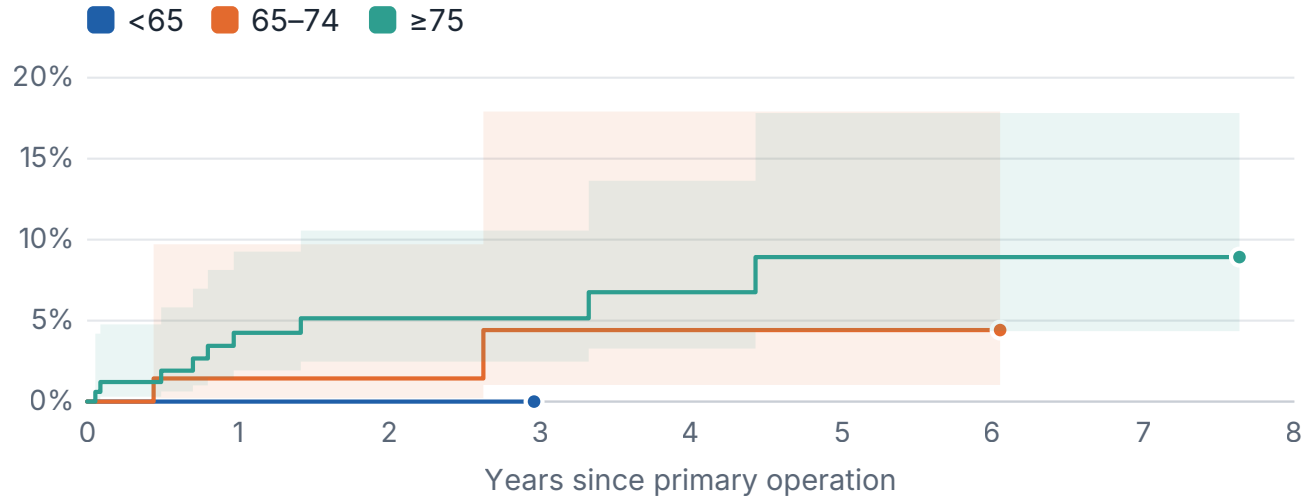


Table 16 **Reasons for revision**

Reason	N	%
Periprosthetic fracture	5	45.5
Infection	3	27.3
Loosening	2	18.2
Other	1	9.1
Total	**11**	**100.0**

Table 17 **Type of revision**

Type	N	%
Major total	6	54.5
Major partial	5	45.5

4 Primary total knee replacement

There were 83 primary total knee replacements in 2025, and 591 in total since 2016. Patients had a mean age of 66.4 years and 69.9% were women. The most common diagnosis was osteoarthritis (86.7%). At 5 years, 4.4% of these joints had been revised. The most common reason for revision was loosening.

Patients in 2025

Table 18 Age and sex, 2025

Age group	Female	Male	Total
<55	20	2	22
55–64	11	5	16
65–74	8	7	15
75–84	15	7	22
≥85	4	4	8
Total	**58**	**25**	**83**

Mean age 66.4 years; median 69.

Table 19 Primary diagnosis

Diagnosis	2025	%	All years	%
Osteoarthritis	72	86.7	523	88.5
Rheumatoid / inflammatory arthritis	4	4.8	30	5.1
Gouty arthritis	5	6.0	27	4.6
Post-traumatic arthritis	2	2.4	11	1.9

Table 20 Fixation

Fixation	2025	%	All years	%
Cemented	77	92.8	561	94.9
Hybrid (cemented femoral / stem)	6	7.2	30	5.1

Table 21 Most used prostheses

Prosthesis	2025	All years
NexGen CR / NexGen tibial	21	127
Triathlon CR / Triathlon tibial	15	120
Attune / Attune tibial	12	88
Legion / Legion tibial	8	88
NexGen LPS / NexGen tibial	11	78
Freedom / Freedom tibial	10	60
Persona / Persona tibial	6	30

Revision outcomes (cumulative percent revision)

Figure 9 Cumulative percent revision of primary total knee replacement

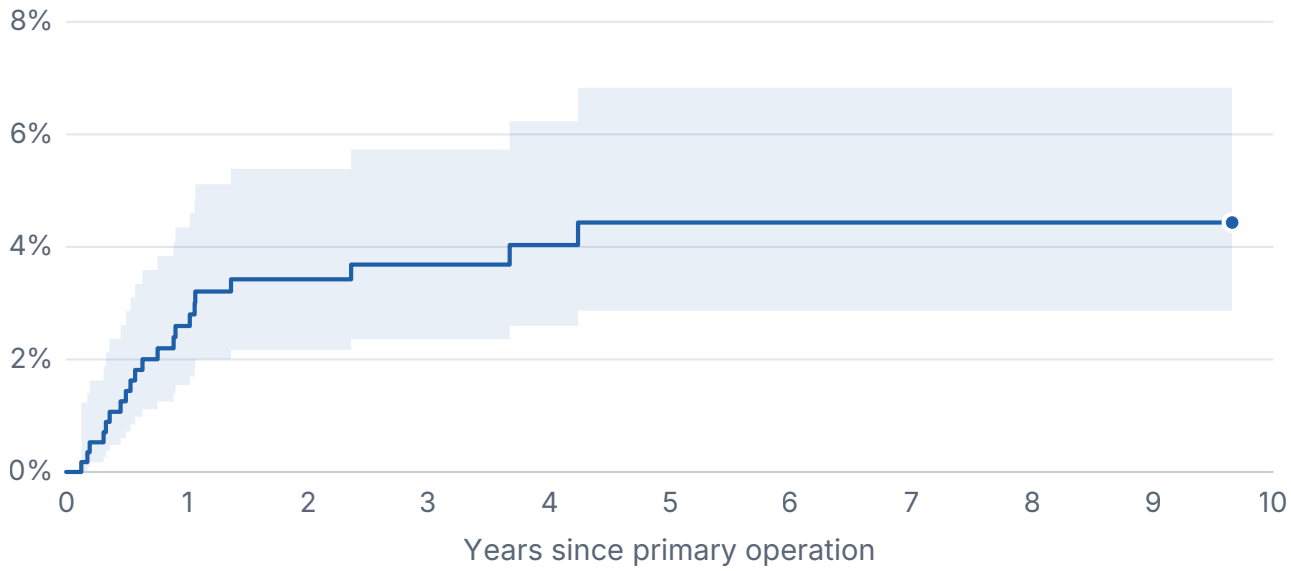


Table 22 Cumulative percent revision of primary total knee replacement (95% CI)

Group	N	N revised	1 yr	3 yrs	5 yrs	10 yrs
All	591	21	2.6 (1.5, 4.3)	3.7 (2.4, 5.7)	4.4 (2.9, 6.8)	—
By fixation						
Cemented	561	20	2.7 (1.6, 4.6)	3.9 (2.5, 6.0)	4.3 (2.8, 6.7)	—
Hybrid (cemented femoral / stem)	30	1	0.0 (0.0, 0.0)	0.0 (0.0, 0.0)	—	—
By age at operation						
<65	273	13	4.0 (2.2, 7.3)	4.9 (2.8, 8.5)	5.6 (3.3, 9.5)	—
65–74	138	2	1.6 (0.4, 6.1)	1.6 (0.4, 6.1)	1.6 (0.4, 6.1)	—
≥75	180	6	1.2 (0.3, 4.7)	3.5 (1.5, 8.3)	5.0 (2.2, 11.4)	—

"—" = not enough patients followed up to that time (fewer than 10 at risk, or fewer than 10 procedures in the group).

Figure 10 **Cumulative percent revision of primary total knee replacement by fixation**

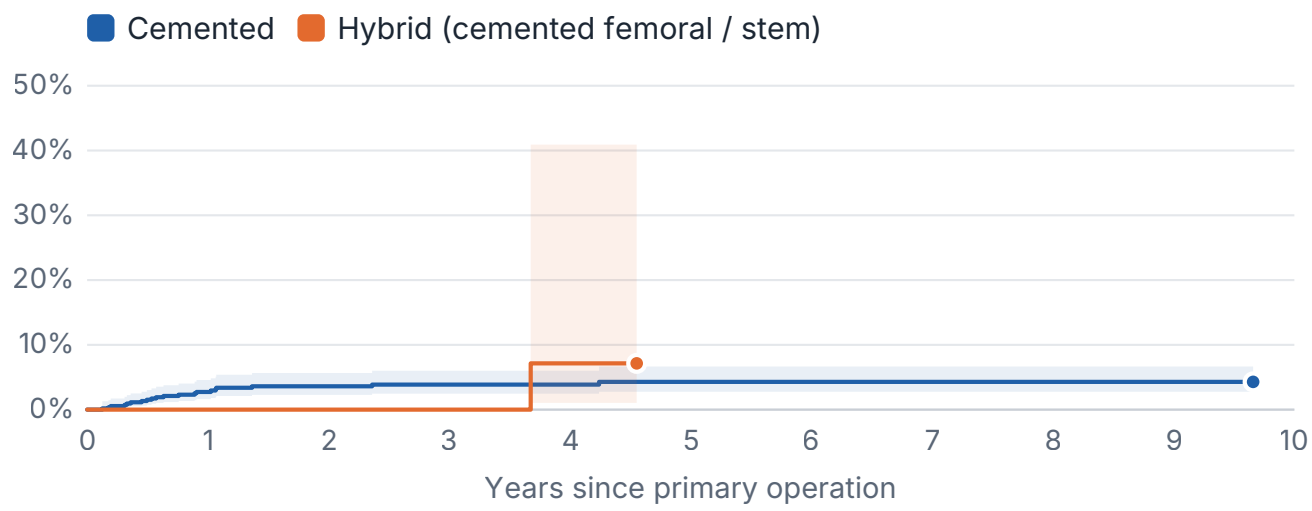


Figure 11 **Cumulative percent revision of primary total knee replacement by age at operation**

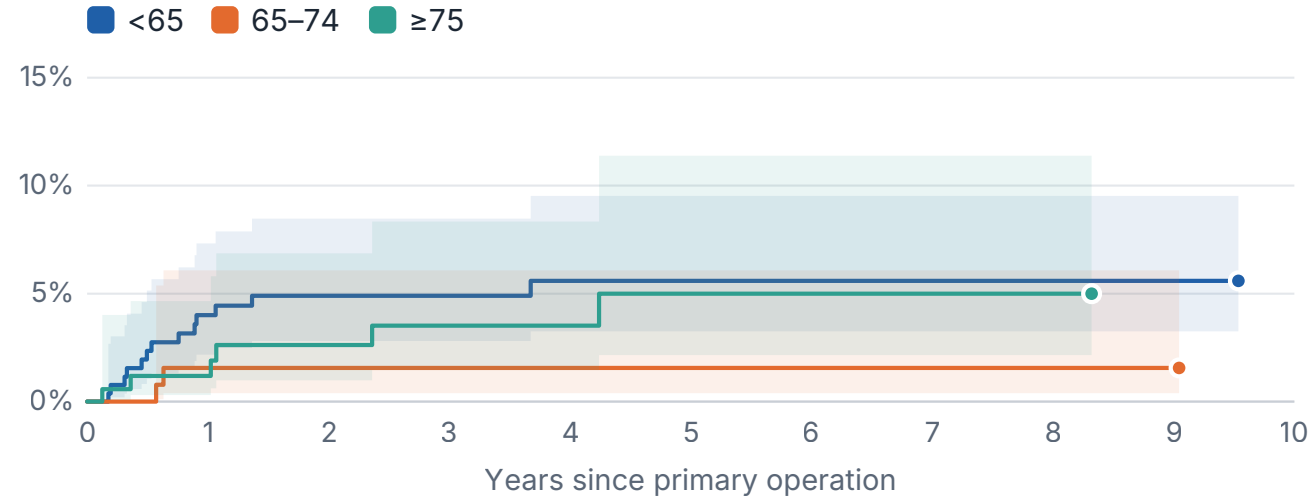


Table 23 **Reasons for revision**

Reason	N	%
Loosening	5	23.8
Infection	5	23.8
Implant failure / breakage	4	19.0
Other	3	14.3
Disease progression (partial joints)	2	9.5
Pain	1	4.8
Periprosthetic fracture	1	4.8
Total	**21**	**100.0**

Table 24 **Type of revision**

Type	N	%
Major total	12	57.1
Minor	8	38.1
Major partial	1	4.8

5 Primary partial knee replacement

There were 9 primary partial knee replacements in 2025, and 53 in total since 2016. Patients had a mean age of 62.0 years and 0.0% were women. The most common diagnosis was osteoarthritis (100.0%). At 5 years, 4.3% of these joints had been revised. The most common reason for revision was loosening.

Patients in 2025

Table 25 Age and sex, 2025

Age group	Female	Male	Total
<55	0	2	2
55–64	0	3	3
65–74	0	3	3
75–84	0	1	1
≥85	0	0	0
Total	**0**	**9**	**9**

Mean age 62.0 years; median 62.

Table 26 Primary diagnosis

Diagnosis	2025	%	All years	%
Osteoarthritis	9	100.0	53	100.0

Table 27 Fixation

Fixation	2025	%	All years	%
Cemented	9	100.0	53	100.0

Table 28 Most used prostheses

Prosthesis	2025	All years
Oxford	9	53

Revision outcomes (cumulative percent revision)

Figure 12 Cumulative percent revision of primary partial knee replacement

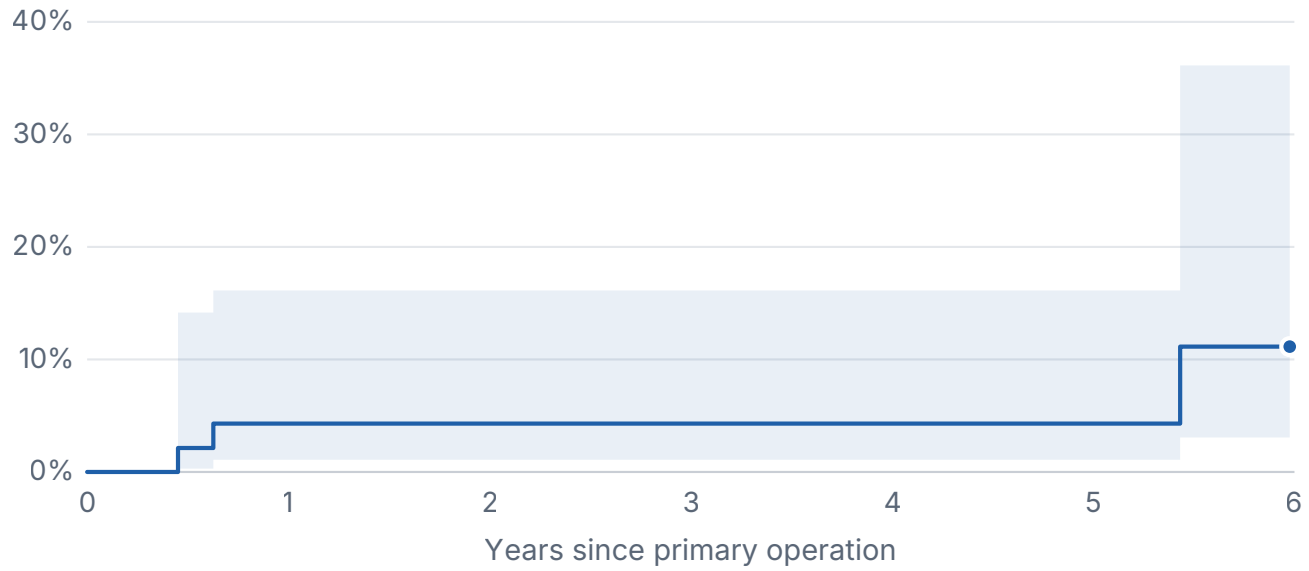


Table 29 Cumulative percent revision of primary partial knee replacement (95% CI)

Group	N	N revised	1 yr	3 yrs	5 yrs	10 yrs
All	53	3	4.3 (1.1, 16.1)	4.3 (1.1, 16.1)	4.3 (1.1, 16.1)	—
By age at operation						
<65	31	1	3.6 (0.5, 22.8)	3.6 (0.5, 22.8)	3.6 (0.5, 22.8)	—
65–74	12	1	—	—	—	—
≥75	10	1	—	—	—	—

"—" = not enough patients followed up to that time (fewer than 10 at risk, or fewer than 10 procedures in the group).

Figure 13 Cumulative percent revision of primary partial knee replacement by age at operation

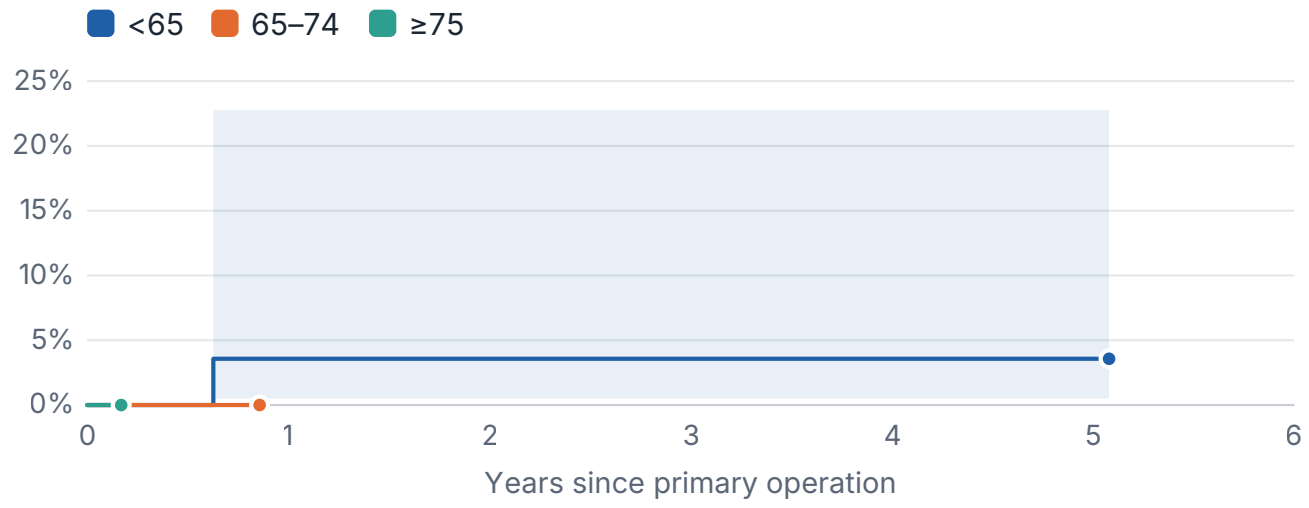


Table 30 Reasons for revision

Reason	N	%
Loosening	2	66.7
Disease progression (partial joints)	1	33.3
Total	**3**	**100.0**

Table 31 Type of revision

Type	N	%
Major total	3	100.0

6 Revision procedures

There were 5 revision procedures in 2025 (5 hip and 0 knee). The most common reason for revision was periprosthetic fracture (80.0%). 100.0% of revisions were linked to a primary procedure in the registry.

This section describes all revision operations recorded in the registry, including revisions of primary procedures performed before the registry began or outside Fiji.

Figure 14 Revision procedures by year

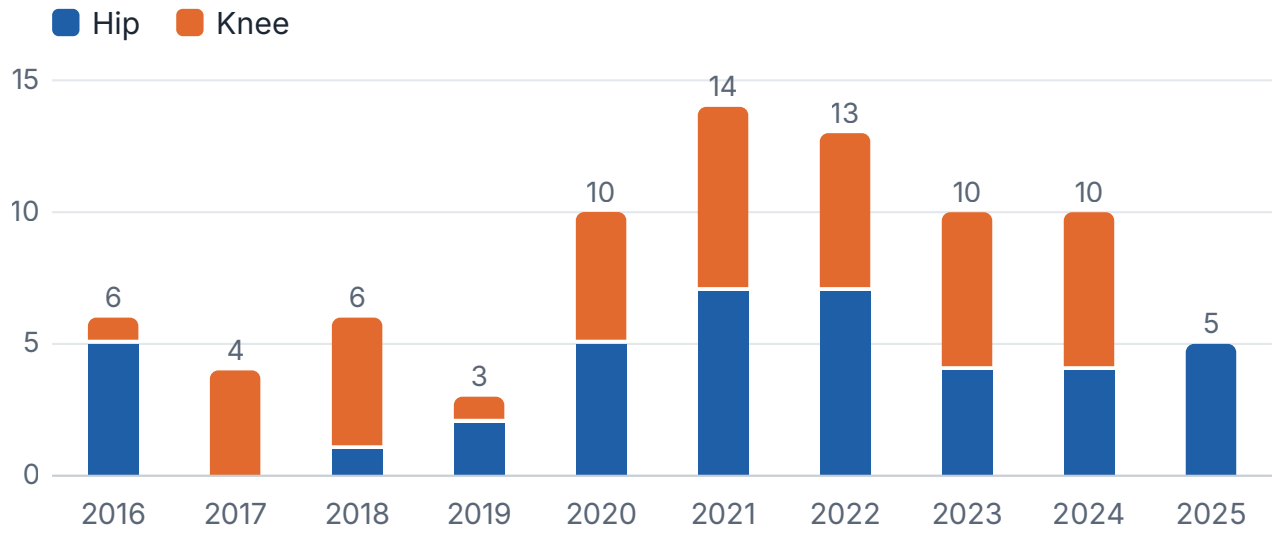


Table 32 Hip revisions by reason

Reason	2025	%	All	%
Loosening	0	0.0	15	37.5
Periprosthetic fracture	4	80.0	9	22.5
Infection	0	0.0	8	20.0
Dislocation / instability	1	20.0	6	15.0
Other	0	0.0	2	5.0

Table 33 Knee revisions by reason

Reason	2025	%	All	%
Loosening	0	0.0	14	34.1
Infection	0	0.0	12	29.3
Implant failure / breakage	0	0.0	5	12.2
Disease progression (partial joints)	0	0.0	3	7.3
Other	0	0.0	3	7.3
Pain	0	0.0	2	4.9
Periprosthetic fracture	0	0.0	2	4.9

Table 34 Components revised (all years)

Component	N	%
Femoral component / stem	64	33.5
Liner / insert	51	26.7
Tibial tray (knee)	29	15.2
Acetabular cup (hip)	25	13.1
Femoral head (hip)	14	7.3
Cement spacer (stage 1)	6	3.1
Patella	2	1.0

A revision can involve more than one component; percentages are of components.

Table 35 Type of revision (all years)

Type	N	%
Major total	53	65.4
Minor	16	19.8
Major partial	12	14.8

Appendix: methods & data quality

How revision rates are calculated

Cumulative percent revision (CPR)

The percentage of primary procedures that have been revised by a given time after the operation (for example, 3.5% at 5 years). This is the main outcome reported by joint replacement registries, including the Australian (AOANJRR) registry.

Kaplan-Meier estimate

CPR is calculated with the Kaplan-Meier method, which allows for patients being followed up for different lengths of time. CPR = 100 × (1 − survival).

Revision

A further operation in which one or more components are added, removed or replaced. Only revisions linked to a primary procedure in the registry count towards that primary's revision rate.

Censoring

Follow-up of each primary procedure ends at the first revision, at the patient's death, or on 31/12/2025 — whichever comes first.

95% confidence interval (CI)

Shown in brackets and as the shaded band on charts; calculated with Greenwood's variance and a log(−log) transformation. Wide intervals mean the estimate is uncertain because numbers are small.

Truncation

Curves and table values stop when fewer than 10 procedures remain at risk, because estimates beyond that point are unreliable.

Major / minor revision

Major total: all main components (femoral and acetabular, or femoral and tibial) replaced. Major partial: one main component replaced. Minor: only parts such as the head, liner, insert or patella.

Small numbers

Fiji's registry is small, so differences between groups should be interpreted with caution. Public versions of these statistics show counts below 5 as “<5”.

Data completeness, 2025

Table 36 Completeness of optional fields

Item	Recorded	Of	%
ASA grade recorded	213	213	100.0
BMI recorded	208	213	97.7
Diabetes status recorded	189	213	88.7
Ethnicity recorded	179	213	84.0
At least one catalogued implant (primaries)	208	208	100.0
Revisions linked to a primary in the registry	5	5	100.0

This report was generated automatically by the FNJRR system from the registry database. Summaries are produced from fixed templates. Surgeons are not identified in this report. **All data in this report are synthetic and for demonstration only.**